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Dr. Thomas Keane, M.D., M.B.A.
Assistant Secretary for Technology Policy &
National Coordinator for Health IT
U.S. Department of Health and Human Services

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care [RIN 0955-AA13]

Dear Secretary Keane,

On behalf of AMGA, we appreciate the opportunity to respond to the Department of Health and Human Services' Request for Information on Artificial Intelligence (AI) in Clinical Care. AMGA represents multispecialty medical groups and integrated systems of care that collectively employ or contract with more than 177,000 physicians and deliver care to more than one in three Americans. Our members are leaders in value-based care delivery and have extensive experience deploying advanced analytics, digital health tools, and early AI-enabled technologies across diverse clinical settings.

AMGA strongly supports HHS's objective of establishing a regulatory, reimbursement, and research posture for AI that is predictable, risk-proportionate, and supportive of innovation while protecting patients, privacy, and public trust. As emphasized in AMGA's prior Health Technology Ecosystem comments, technology policy is most effective when it reflects the operational realities of clinical care and reduces administrative burden. AI adoption will succeed only if it is integrated into aligned payment models, interoperable infrastructure, and clinically grounded governance frameworks.

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

AMGA members report that the most significant barriers to private-sector innovation and adoption of AI in clinical care stem from misalignment between AI's value proposition and existing health care policy structures. Fee-for-service payment systems, in particular, do not adequately reward the preventive, longitudinal, and population-level benefits that AI tools are designed to deliver. As a result, providers often struggle to justify the substantial upfront investments required to deploy AI responsibly, integrate it into clinical workflows, and maintain it over time.

Regulatory fragmentation further compounds this challenge. Overlapping oversight across CMS,

ONC, and OCR creates uncertainty regarding applicable standards, compliance obligations, and enforcement risk—particularly when AI tools are embedded within EHRs or used as clinical decision support. In addition, AI development and performance are constrained by incomplete and inconsistent access to data, including limited availability of commercial and Medicare Advantage claims data and variability in interoperability implementation across certified health IT systems.

Administrative burden also remains a persistent barrier. Duplicative documentation requirements, quality reporting obligations, and program-specific compliance processes divert clinician and organizational resources away from innovation and care improvement.

State-based licensure requirements restrict the scalability of AI-enabled telehealth and remote clinical support models, while rising cybersecurity risks impose additional operational and financial costs that are not adequately recognized or offset under current payment and regulatory frameworks.

Finally, recent CMS Innovation Center models, such as the WISeR model, further illustrate provider concerns regarding the use of AI-enabled tools in utilization management and prior authorization decision-making. WISeR characterizes certain services as “low value” despite those services having already undergone rigorous review through National Coverage Determinations (NCDs) and Local Coverage Determinations (LCDs). While CMS states that the model does not directly modify NCDs or LCDs, it effectively re-scrutinizes established coverage policies and raises concerns about duplicative oversight and the potential for informal policy shifts outside the traditional rulemaking process. Such approaches risk undermining trust in evidence-based coverage frameworks that providers rely upon for clinical certainty.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care?

To incentivize effective and responsible AI use in clinical care, AMGA urges HHS to prioritize modernization of payment and regulatory frameworks so they better reflect how care is delivered and how AI creates value. Payment models should reward prevention, care coordination, and population health management—areas where AI tools have demonstrated the greatest potential to improve outcomes and reduce costs.

Extending and stabilizing incentives for participation in Advanced Alternative Payment Models, including the five percent APM bonus, would provide providers with the financial certainty necessary to make long-term investments in AI-enabled infrastructure. HHS should also allow AI-enabled tools and digital infrastructure to count as allowable expenditures in ACO benchmarking and financial reconciliation methodologies.

Additionally, certification and reporting requirements that force costly EHR upgrades without clear clinical or patient benefit should be reevaluated to ensure they do not inadvertently stifle innovation.

As AI tools are increasingly incorporated into coverage, utilization management, and prior authorization processes, HHS should establish clear guardrails to ensure transparency, accountability, and alignment with established coverage policies. The wide concerns around the WISeR model relate back to this fundamental issue. When AI is used to influence payment

decisions, the standards governing its use should be clear, predictable, and subject to appropriate rulemaking safeguards.

3. For non-medical devices, what novel legal and implementation issues exist, and what role should HHS play?

The use of AI in clinical care, particularly when deployed through non-medical devices or third-party applications, raises novel legal and implementation challenges. These include uncertainty regarding liability and accountability when AI tools are embedded in EHRs or used as clinical decision support, as well as contractual ambiguity surrounding responsibility for AI-generated recommendations. AI also amplifies privacy and cybersecurity risks due to the scale, sensitivity, and aggregation of data involved, creating new compliance challenges at the intersection of HIPAA, information blocking, and patient safety requirements.

AMGA believes HHS has an important role to play in clarifying governance expectations for AI-enabled clinical decision support tools and supporting safe harbors for providers who use AI responsibly within established clinical workflows. OCR privacy enforcement should be aligned with the realities of AI-driven care delivery, and cybersecurity readiness should be recognized as a foundational requirement for safe AI deployment rather than an optional or ancillary consideration.

Models such as WISer also raise important legal and implementation questions regarding accountability when AI tools are used to support or inform prior authorization determinations. Even if AI is positioned as an advisory tool, its operational integration into payment review processes can materially affect patient access and provider reimbursement. HHS should clarify how responsibility is allocated among technology vendors, CMS contractors, and providers when AI-generated recommendations contribute to denials, delays, or downstream clinical consequences.

4. For non-medical devices, what AI evaluation methods and metrics are most promising, and should HHS further support them?

AMGA supports AI evaluation approaches that prioritize real-world clinical impact rather than technical performance alone. Effective evaluation should focus on patient outcomes, workflow integration, clinician usability, equity, and patient experience. Enterprise-level, outcome-based measures—such as those aligned with CMS’s Universal Foundation and the AMGA Value Measure Set—are particularly well suited to assessing AI’s contribution to coordinated, team-based care.

HHS should further support these evaluation efforts through pilot programs, demonstration projects, and implementation guides that test AI tools in diverse care settings. Grants, cooperative agreements, and learning collaboratives focused on implementation science would help ensure that AI tools are deployed safely and effectively. Importantly, guardrails against AI bias and misuse should be embedded within evaluation frameworks, including ongoing monitoring of performance across patient populations and transparency regarding limitations and appropriate use.

5. How can HHS best support private-sector accreditation, certification, testing, and credentialing?

AMGA recommends that HHS support private-sector accreditation and certification by

developing modular, function-specific certification pathways for AI-enabled tools. A parallel certification track focused on value-based and population health capabilities would better reflect the operational needs of providers engaged in advanced care models. Certification approaches should avoid forcing full EHR replacement or unnecessary system upgrades and should recognize third-party validation and attestation pathways, particularly for small and rural providers with limited resources.

6. Where have AI tools deployed in clinical care met or exceeded expectations, and where have they fallen short?

AI tools have shown meaningful promise in areas such as risk stratification, identification of rising-risk patients, care gap identification, proactive outreach, remote patient monitoring, and chronic disease management. When supported by timely and complete data, AI-enabled analytics can also enhance population health management by integrating claims and clinical information into actionable insights.

However, AI tools often fall short when claims data is incomplete or delayed, interoperability standards are inconsistently implemented, or payment models fail to reward avoided utilization and downstream savings. Inadequate data quality or lack of representativeness can also limit AI effectiveness and raise concerns about equity and reliability.

AI tools are most effective when they enhance care coordination and proactive population management. They are more likely to generate provider concern when deployed primarily as cost-containment or utilization control mechanisms without transparent guardrails. The WISeR model illustrates this tension, particularly where prior authorization processes intersect with services already deemed reasonable and necessary under existing Medicare coverage policies.

7. Which roles or governing bodies most influence AI adoption, and what administrative hurdles exist?

Within health care organizations, AI adoption decisions are typically influenced by clinical leadership, compliance and legal teams, IT and cybersecurity leadership, and finance and population health leaders. Administrative hurdles include regulatory uncertainty, unclear return on investment under fee-for-service payment, integration challenges with existing EHRs and workflows, and limited internal resources. These challenges are particularly acute for small and rural practices.

8. Where would enhanced interoperability accelerate AI development and adoption?

Enhanced interoperability is foundational to AI development and adoption in clinical care. Standardized FHIR APIs across all certified EHRs, timely access to Medicare Advantage and commercial claims data via APIs, and enforceable standards for data completeness, timeliness, and usability under TECA would significantly improve AI performance and scalability. The development of a national FHIR-based provider directory would further support attribution, contracting, and care coordination while reducing administrative burden.

9. What challenges do patients and caregivers want AI to address, and what concerns do they have?

Patients and caregivers want AI to improve care coordination, enable earlier intervention, support personalized care, and reduce delays in accessing quality care. At the same time, they have legitimate concerns about losing clinician judgment or human oversight, and the privacy

and security of their health data. Maintaining trust will require that AI tools augment—not replace—the clinician-patient relationship.

10. Are there specific areas of AI research HHS should prioritize?

AMGA recommends that HHS prioritize AI research focused on early risk identification, preventive interventions, integration of patient-reported outcomes and longitudinal data, and reduction of administrative burden and clinician burnout. Implementation science should be emphasized to support safe, scalable deployment across care settings. Research should assess real-world outcomes, equity impacts, and the distribution of costs and benefits across patients, providers, and payers. AI bias mitigation and misuse prevention should be explicit research priorities to ensure that AI tools improve care equitably across populations.

Conclusion

AMGA appreciates HHS’s leadership in advancing a clear, predictable, and risk-proportionate framework for AI in clinical care.

AI has significant potential to improve patient outcomes, reduce administrative burden, and support the transition to value-based care, but this promise can only be realized if regulatory, payment, and interoperability policies are aligned with real-world clinical practice. AMGA and its members stand ready to continue partnering with HHS to advance responsible, patient-centered AI adoption.

Should you have questions, please do not hesitate to contact Darryl M. Drevna, Senior Director of Regulatory Affairs, at 703.838.0033 ext. 339 or at ddrevna@amga.org.

Sincerely,

A handwritten signature in cursive script, reading "Jerry Penso".

Jerry Penso, MD, MBA
President and Chief Executive Officer, AMGA